AM-05 Hypotension / Shock

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for adults with SBP below 90 mmHg or clinical evidence of inadequate perfusion regardless of BP. Identify and correct the cause while supporting perfusion.

Cause-directed shock and perfusion pathway

1. Recognize shock	SBP below 90 or inadequate perfusion: mental-status change, weak pulses, cool/mottled skin, delayed refill, or progressive deterioration.
2. Immediate support	Airway/ventilation, oxygen when indicated, hemorrhage control, prevent heat loss, monitoring, glucose, IV/IO, and rapid transport.
3. Identify cause	Hemorrhagic, septic, hypovolemic, cardiogenic, obstructive, anaphylactic, toxicologic, adrenal, or obstetric.
4. Fluid when appropriate	Nonhemorrhagic shock: isotonic crystalloid 500 mL IV/IO and reassess. Use 250 mL aliquots if fluid-overload risk.
5. Stop fluid	Stop when perfusion improves, SBP about 90-100, MAP at least 65, pulmonary edema develops, respiratory status worsens, or fluid is unlikely to help.
6. Cause-specific goals	Sepsis: up to 30 mL/kg only while fluid responsive/no pulmonary edema. Hemorrhage without TBI: minimum fluid for radial pulse, mentation, or SBP about 80-90. TBI: SBP at least 110.
7. Persistent shock	Norepinephrine 0.05-0.5 mcg/kg/min IV/IO, titrated to perfusion and MAP at least 65. Start earlier when further fluid is contraindicated.
8. Alternative	If norepinephrine is unavailable or significant bradycardia accompanies shock: epinephrine infusion 2-10 mcg/min IV/IO, titrated to perfusion and MAP at least 65.

Key treatment and disposition

Item	Approved action
Pregnancy	Beyond about 20 weeks: left uterine displacement/left lateral positioning when feasible; follow AO-03 for pregnancy-related causes.
Cause control	Control bleeding, decompress tension pneumothorax, treat anaphylaxis, and correct causative dysrhythmia without waiting for BP normalization.

The treatment goal is adequate perfusion, not normalization of blood pressure. Fluids and vasopressors do not replace correction of the cause.

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SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Do not use indiscriminate large-volume crystalloid in uncontrolled hemorrhage, pulmonary edema, cardiogenic shock, or suspected volume overload.

- Do not use permissive hypotension when traumatic brain injury is suspected.

- Use continuous ECG and frequent BP reassessment for vasopressors. Do not delay transport for a fluid target or repeated IV attempts.

- Push-dose epinephrine is only a temporary bridge; use the dedicated medication reference and begin an infusion when indicated.

Linked pathways

Pathway	Use when
AM-01	Anaphylactic shock
AM-03	Renal failure and fluid-risk guidance
AC-04	Ischemia/infarction causing shock
AC-05	Pulmonary edema
TB-06	Traumatic shock/hemorrhage
AM-07	Peri-arrest deterioration

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.